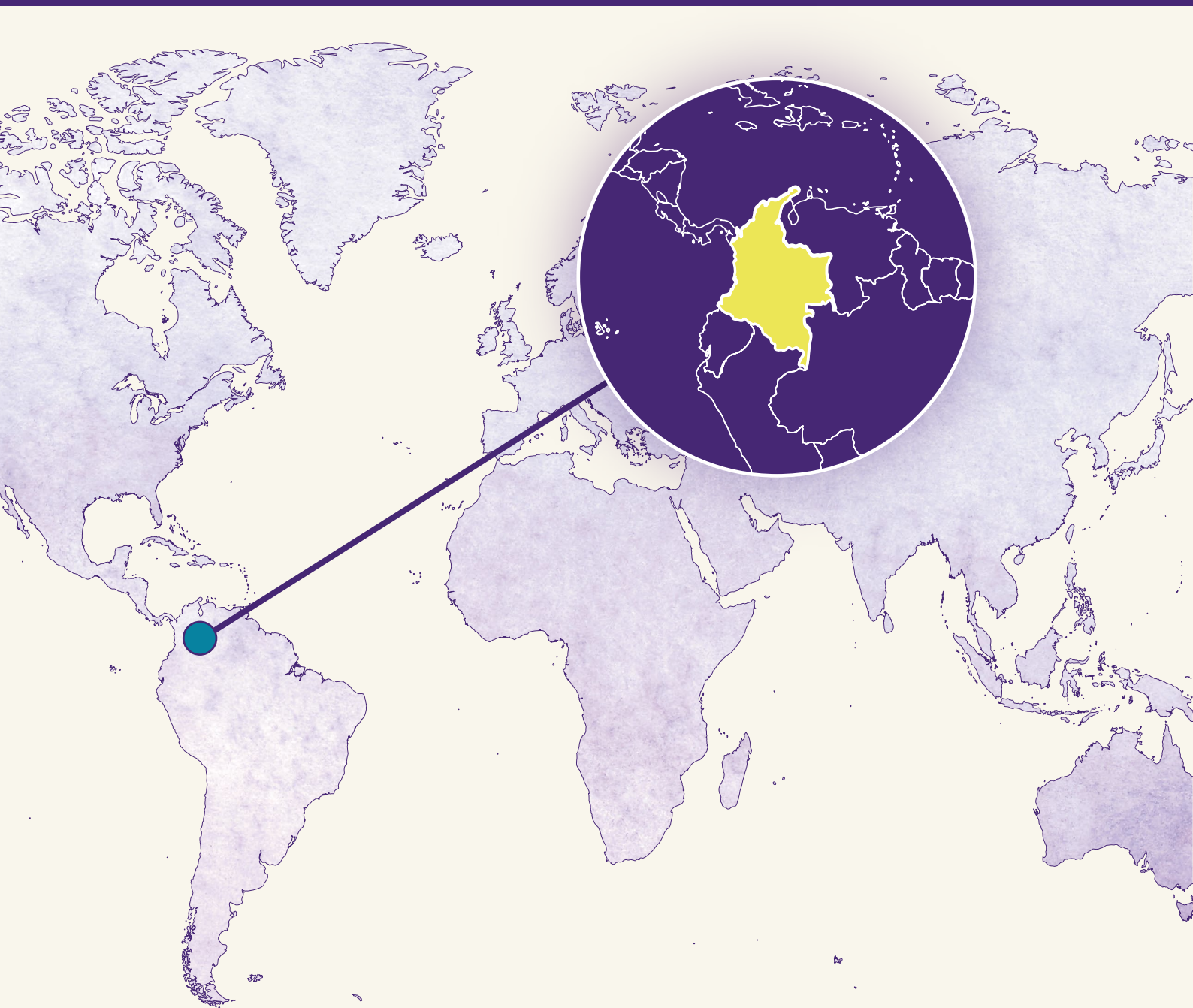


COLOMBIA

Violence Against Health Care in Conflict 2025



**SAFEGUARDING
HEALTH
IN CONFLICT**



**Insecurity
Insight**
Data on People in Danger



REPORTED INCIDENTS AND MOST COMMONLY REPORTED CONCERNS

 REPORTED INCIDENTS	 INCIDENTS WHERE HEALTH FACILITIES WERE ATTACKED	 HEALTH WORKERS KILLED	 HEALTH WORKERS KIDNAPPED
2025			
28	10	7	15
2024			
19	7	4	7
2023			
18	1	9	4

Source: Insecurity Insight for the SHCC.

OVERVIEW

Using data collected by Insecurity Insight, the Safeguarding Health in Conflict Coalition (SHCC) identified 28 incidents of violence against or obstruction of health care in Colombia in 2025, compared to 19 in 2024 and 18 in 2023.¹ Among incidents reported in 2025, health facilities were attacked ten times, seven health workers were killed and were 15 kidnapped.

The actual number of incidents and the severity of the problem are likely greater, because attacks on health care may not have been reported in some locations and conflict-related service closures may not have been reflected in the data, while cuts to United States Agency for International Development (USAID) funding further reduced the presence of health care providers and the operation of health facilities.

Methodology and sources

Information on incidents of violence against health care in Colombia are compiled from information-sharing projects and open sources. Data is continuously updated and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care. See [Methodology](#) for further information.



THE CONTEXT

In 2025, Colombia experienced increased activities by armed groups. Nearly a decade since the 2016 peace accord between the government and the Revolutionary Armed Forces of Colombia (FARC), the estimated number of members in armed groups quadrupled from 6,500 in 2017 to more than 25,000 in 2025. Violence among local parties involved the FARC; the Gulf Clan; the National Liberation Army (ELN), including fighters from the Manuel Vásquez Castaño Front of the Eastern War Front; and FARC dissident factions, including Hernando Gonzalez Acosta. The use of explosive weapons, including drone-delivered explosives and landmines, increased in 2025, with at least 544 civilians affected by the use of explosive devices between January and August, representing a 145% increase compared to the same period in 2024. Violence increased in the departments of Antioquia, Cauca, Norte de Santander, and Valle del Cauca, fueled by disputes over drug-trafficking routes, illegal mining, and territorial control.

Displacement surged, particularly in western departments and Norte de Santander. Between January and August, over 79,500 people were reportedly displaced, a 94% increase from 2024. In August 2025 alone, at least 3,112 people were displaced, mainly in the departments of Bolívar, Cauca and Chocó. Over 137,000 people were confined – a practice used by armed groups to restrict civilian movement and exert territorial control – and 51% of the affected population belonged to Indigenous and Afro-Colombian communities. The most affected departments were Bolívar, Cauca, Chocó, Guaviare and Valle del Cauca.



Armed groups' forced recruitment of civilians, including children, remained a concern in 2025, with over 578 cases of child recruitment reported in the first half of the year. In addition, at least 15 children were killed during four security forces operations against FARC dissidents in Amazonas, Arauca, and Guaviare departments between August and November 2025. Attacks, threats, and movement restrictions imposed by armed actors continued to affect health care delivery, particularly in rural and marginalized communities, despite the Colombian health care system being among the strongest in Latin America.



USAID funding freeze endangered the 2016 peace deal by withdrawing up to USD 440 million annually from around 80 programs, including those focusing on cocaine production, land reform and movement towards a transitional justice system, thus increasing the risk of violence. Organizations aiding human rights, democracy, peacebuilding, and helping Indigenous and Afro-Colombian people were affected.

VIOLENCE AGAINST OR OBSTRUCTION OF HEALTH CARE IN 2025

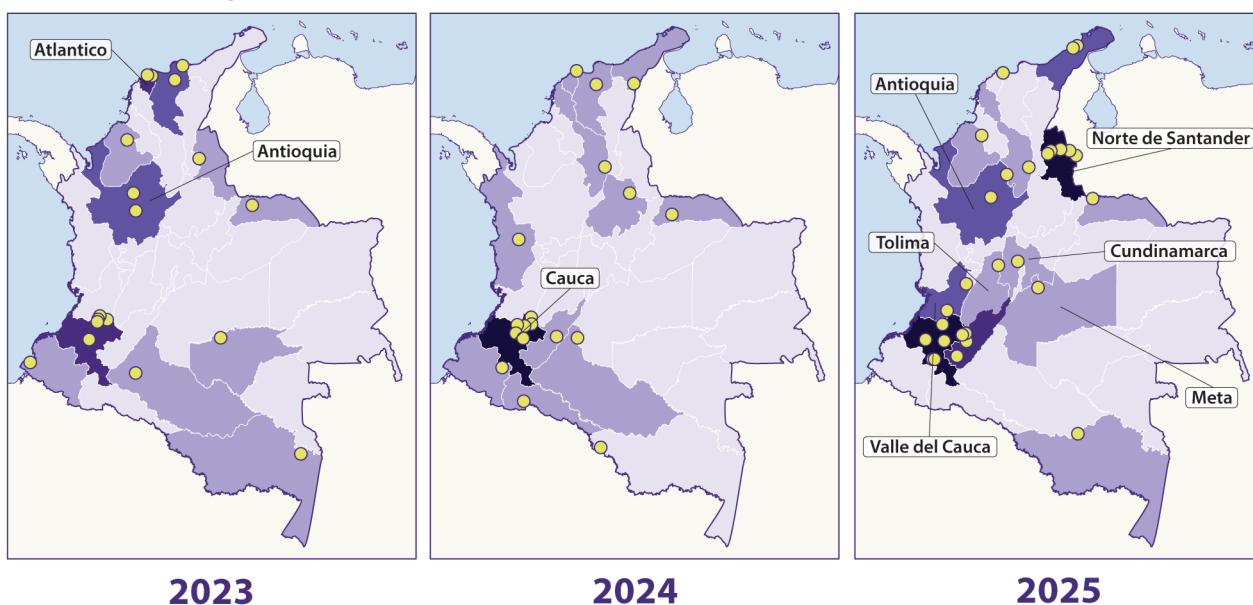
In 2025, publicly reported violence against or obstruction of health care in Colombia increased by nearly 50%, with incidents reported across 16 departments – four more affected areas than in 2024.² All reported cases affected health care providers working for national health structures. Attacks on health facilities and the killing of health workers increased, while kidnappings of health workers doubled.

As in previous years, most incidents in 2025 were recorded in the Cauca department, with small shifts from Colombia's peripheral departments, such as Caquetá, Chocó, Magdalena, Nariño, Putumayo, and Santander, toward more central Andean and plains areas, including Antioquia, Cundinamarca, Meta, Norte de Santander, Tolima, and Valle del Cauca. Incidents continued in Atlántico, Arauca, Amazonas, Bolívar Huila and La Guajira in 2025.

The majority of reported violence impacting health care was attributed to unidentified men with guns, likely affiliated to armed groups operating in the area in question. Armed violence attributed to FARC dissidents impacting health care increased from one recorded case in 2024 to four in 2025. On two occasions, FARC dissidents' armed drones impacted health care in Cauca.³ Hernando Gonzalez Acosta FARC dissidents kidnapped eight health workers and UN mission personnel while they were traveling in Huila department.⁴

Known locations of reported incidents affecting health care in Colombia, 2023-2025

As in previous years, incidents in 2025 were concentrated in Cauca department, where violence remains persistent, but shifted slightly from Colombia's peripheral regions toward central departments such as Antioquia, Cundinamarca, Meta, Norte de Santander, Tolima, and Valle del Cauca.



Source: Insecurity Insight for the SHCC.



ELN members, including fighters from the Manuel Vásquez Castaño Front of the Eastern War Front, abducted a patient from a medical facility after clashes with the Guadalupe Salcedo 10th Front, intercepted an ambulance and injured two health workers while abducting another patient, and killed a health worker in crossfire during an attack on a police station.⁵

Colombian military forces dismantled a clandestine medical facility operated by the Gulf Clan in Antioquia and seized all the medicines it contained.⁶

Health worker killings and kidnappings

Fifteen health workers were reported killed in seven incidents in 2025, compared to four in four incidents in 2024 and nine in eight incidents in 2023. Victims in 2025 included an ambulance driver, a nurse, and optometrists killed in shootings at medical facilities. Unidentified armed actors killed victims inside their homes and while they were traveling to provide health care. An ambulance driver was shot and killed in crossfire during an ELN attack on a police station in Norte de Santander.⁷

Kidnappings by armed actors also continued in 2025, with 15 health workers kidnapped in six incidents, compared to seven in three incidents in 2024 and three in three incidents in 2023. A nurse was taken from her home in Norte de Santander and a health worker was kidnapped along with her children, who were later released.⁸ Armed groups forced a doctor to travel with them to provide medical care to their members, while another health worker was kidnapped after being lured to attend a bogus workshop.⁹ Criminals also targeted medical transport, kidnapping drivers and assistants and stealing medicines, although some victims were later rescued and some supplies were recovered.¹⁰ Ambulances were intercepted and health workers were detained for hours.¹¹ Four of the 15 kidnapped health workers were released, but the fates of the remaining staff members remain unclear.

Health facilities attacked

Health facilities, including children's hospitals and field clinics, were attacked at least ten times in 2025, compared to seven incidents in 2024 and one publicly reported incident in 2023. These attacks occurred across nine departments and often involved shootings, the use of explosive devices, and forced entry. Patients were abducted and staff were threatened and/or harmed.

In addition to the incidents at health facilities attributed to the ELN, FARC, and Colombian military personnel, unidentified actors armed with firearms entered health facilities, stole medical supplies, and robbed and threatened patients; shot guards and people leaving facilities; attacked clinics with gunfire; and entered facilities and assaulted patients receiving care.



This chapter is based on 2023-2025 COL SHCC Health Care Data. Download this data on the [Humanitarian Data Exchange \(HDX\)](#).

Insecurity Insight updates data continuously and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care.



THE IMPACT OF ATTACKS ON HEALTH CARE

Escalating violence between armed groups in 2025 severely undermined access to health care in parts of Colombia. The [ICRC](#) reported that in areas where people cannot move freely, access to primary health services deteriorated sharply, increasing stress, mistrust and social fragmentation. Pre-existing structural inequalities further constrained health care access for internally displaced persons (IDPs). While humanitarian organizations provided [limited](#) health care services in some departments, including Arauca, Chocó, La Guajira, and Norte de Santander, [many IDPs remained without access to health care](#).

Health facilities and services faced increased disruptions, including the suspension of their activities. Following a surge in violence between multiple armed groups in the Ábrego, Teorama, and Tibú municipalities in the Catatumbo region's Norte de Santander department, at least four basic health care centers [closed](#) or suspended their services, while outreach medical services were [halted](#). Vaccination programs, nutritional services and mental health services were interrupted, while only limited emergency services remained operational. Disruptions of this kind can leave entire communities without access to preventive and primary health care, increasing the risk of untreated illness and long-term negative impacts on people's health status.

Women and girls were particularly vulnerable to disruptions to health services. In early 2025, Médecins Sans Frontières (MSF) reported a deterioration in community health conditions during its emergency response in the Catatumbo region of Norte de Santander department. Children displayed symptoms of malnutrition, while many pregnant women could not access prenatal care, increasing risks for both maternal and infant health. Some patients showed severe violence-related psychological symptoms that were consistent with a recent [survey study](#) by Østergaard et al. linking repeated exposure to armed conflict and its impact on health, including mental health. Armed groups' [recruitment and exploitation of women and girls](#), coupled with restricted health care and reproductive services, further put women and girls at risk.

Renewed government [commitments](#) to strengthen primary health care in rural and conflict-affected areas, together with [progress in various health indicators](#), represented key steps toward more equitable nationwide health care access.



- 1 The SHCC's 2024 Annual Report cited 18 incidents of violence against or obstruction of health care in Colombia in 2024, compared to 18 in 2023 and 13 in 2022. Data is continuously updated and numbers can change if new information is made publicly available.
- 2 In October 2025, the website of the Ombudsman's Office declared over 325 registered cases of violence against health care, although the description of these events is not public and depends on confidential information obtained through a national system managed by the Colombian government.
- 3 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident numbers 122289; 122296.
- 4 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident number 119663.
- 5 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident numbers 96309; 119994; 123613.
- 6 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident number 119531.
- 7 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident number 123613.
- 8 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident numbers 123166; 116636.
- 9 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident numbers 119662; 122294.
- 10 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident number 122298.
- 11 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 COL SHCC Health Care Data. Incident number 122288.

SAFEGUARDING HEALTH IN CONFLICT

The Safeguarding Health in Conflict Coalition is a group of more than 40 organizations working to protect health workers and services threatened by war or civil unrest. We have raised awareness of global attacks on health and pressed United Nations agencies for greater global action to protect the security of health care. We monitor attacks, strengthen universal norms of respect for the right to health, and demand accountability for perpetrators. Suggested citation: Safeguarding Health in Conflict Coalition (SHCC) 2026. Care in the Crosshairs: Violence Against Health Care in Conflict, May 2026. <https://shcc.pub/SHCCAnnualReport2025>
<https://safeguarding-health.com>

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